

PAEDIATRIC EMERGENCY DEPARTMENT,
HOSPITAL TUNKU AZIZAH
PAEDIATRIC SEIZURE ORDER SET

DATE : _____	TIME: _____	AGE: _____ ☐ < 6 month old ☐ ≥ 6 month old till ≤ 18 year old	WEIGHT: _____
NAME: _____			
IC: _____	RN: _____	MALE / FEMALE	

A) HISTORY

COMORBIDS (can tick more than 1 box) <i>*If any COMORBIDS is present, PLEASE TAKE SERUM CALCIUM, MAGNESIUM</i>	☐ Previous history of hypocalcaemia ☐ Previous history of hypomagnesaemia ☐ Known Chronic Kidney disease ☐ End Stage Renal Failure ☐ Known Vitamin D deficiency ☐ Chronic pancreatitis ☐ Chronic diarrhoea ☐ Malabsorption
PRESENTING COMPLAINT <i>*If the answer</i> <i>"Yes" Either Question 1 or Question 2</i> AND <i>"No" For Both Question Number 4 and 5</i> <i>PLEASE TAKE SERUM CALCIUM, MAGNESIUM</i>	1. Is it patient actively seizing upon arrival or while in PED HTA? ☐ Yes ☐ No 2. Does patient develop status epilepticus ☐ Yes ☐ No 3. Semiology ☐ Generalized ☐ Focal ☐ Absence ☐ Other: _____ 4. Does patient has fever ? ☐ Yes ☐ No 5. Does patient has underlying epilepsy / recurrent afebrile seizure? ☐ Yes ☐ No

B) PHYSICAL EXAMINATION

1. AVPU	☐ Alert ☐ Verbal ☐ Pain ☐ Unresponsive				
2. PUPIL	☐ Equal ☐ Unequal <table style="width: 100%;"> <tr> <td style="width: 50%;"> Right side ☐ Reactive ☐ Sluggish ☐ Not reactive </td> <td style="width: 50%;"> Left t side ☐ Reactive ☐ Sluggish ☐ Not reactive </td> </tr> <tr> <td colspan="2">Size : Right mm / Left mm</td> </tr> </table>	Right side ☐ Reactive ☐ Sluggish ☐ Not reactive	Left t side ☐ Reactive ☐ Sluggish ☐ Not reactive	Size : Right mm / Left mm	
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Size : Right mm / Left mm					
3. VITAL SIGNS	BP: HR: RR: T: Spo2: DXT:				

4.CENTRAL NERVOUS SYSTEM EXAMINATION	<table border="1" style="width: 100%; border-collapse: collapse; text-align: center;"> <tr> <td style="width: 15%;"></td> <td colspan="2" style="width: 40%;">Upper limb</td> <td style="width: 5%;"></td> <td colspan="2" style="width: 40%;">Lower limb</td> </tr> <tr> <td></td> <td style="width: 15%;">Right</td> <td style="width: 15%;">Left</td> <td></td> <td style="width: 15%;">Right</td> <td style="width: 15%;">Left</td> </tr> <tr> <td style="text-align: left;">Tone</td> <td></td> <td></td> <td></td> <td></td> <td></td> </tr> <tr> <td style="text-align: left;">Power</td> <td></td> <td></td> <td></td> <td></td> <td></td> </tr> <tr> <td style="text-align: left;">Reflex</td> <td></td> <td></td> <td></td> <td></td> <td></td> </tr> <tr> <td style="text-align: left;">Clonus</td> <td></td> <td></td> <td></td> <td></td> <td></td> </tr> <tr> <td style="text-align: left;">Babinski</td> <td></td> <td></td> <td></td> <td></td> <td></td> </tr> </table> (i)Brudzinski's sign:Yes/No (ii)Kernig's sign:Yes /No (iii)Cerebellar sign: Positive/ Negative <i>*please circle ONLY ONE for question No (i) , (ii) and (iii)</i>		Upper limb			Lower limb			Right	Left		Right	Left	Tone						Power						Reflex						Clonus						Babinski					
	Upper limb			Lower limb																																							
	Right	Left		Right	Left																																						
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Babinski																																											
DIAGNOSIS																																											
INVESTIGATION	☐ Calcium ☐ Full blood count ☐ Renal profile ☐ Others relevant investigation _____ ☐ Magnesium																																										
MANAGEMENT	<u>ONGOING SEIZURES , > 5 MINUTES</u> If difficult or no IV access ☐ IN Midazolam 0.2 mg/kg (max 10 mg) ☐ Buccal Midazolam 0.2-0.5 mg/kg (max 10 mg) ☐ Supp Diazepam 0.2 -0.5 mg/kg (max 10 mg) or IV line present ☐ IV Midazolam 0.2 mg/kg slow bolus (at 2mg/min, max 10 mg) ☐ IV Diazepam 0.2 mg/kg slow bolus (at 2mg/min, max 10 mg) <u>SEIZURES CONTINUE > 5 MINUTES AFTER 2ND DOSE OF BENZODIAZEPINE</u> ☐ IV Phenytoin 20 mg/kg (max loading dose 1.25g) Dilute in 0.9% saline, maximum concentration at 10 mg/ml Infuse over 20-30 mins with cardiac monitoring <i>* If contraindicated to phenytoin ,may use IV Phenobarbitone</i> <u>SEIZURES CONTINUE > 10 MINUTES AFTER PHENYTOIN</u> <i>*Consult Paediatrician</i> ☐ IV Midazolam 0.2 mg/kg bolus (at 2 mg/min; max 10 mg) ☐ IV Phenobarbitone 20 mg/kg (max loading dose 1 g) ☐ IV Levetiracetam 40 mg /kg infused over 10 minutes , then 20 mg/kg 12 hourly ☐ IV Sodium Valproate 20 mg/kg (max loading 1.25 g, given over 1-5 mins, at 20-50 mg/min), then infusion 1-5 mg/kg/hour for 6-12 hours)																																										
DISPOSITION	☐ Admit ☐ Discharge																																										

Medical Officer in charge: _____
 (sign and Cop)